SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Nivolumab + ipilimumab · colorectal after chemotherapy

MSI-H/dMMR metastatic colorectal cancer after fluoropyrimidine-based combination chemotherapy · NCCP 00551 · version 7a · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
These two immunotherapy medicines aim to control bowel cancer with a confirmed MSI-H/dMMR DNA-repair feature. This pathway follows previous fluoropyrimidine-based combination chemotherapy.

Schedule
Cycles 1–4: nivolumab 3 mg/kg, then ipilimumab 1 mg/kg, on Day 1 every 21 days. The first nivolumab-only dose is due three weeks after the final combination dose, subject to clinical review. Nivolumab alone: 240 mg IV or 600 mg under the skin every two weeks, or 1200 mg under the skin every four weeks.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Nivolumab
IMMUNOTHERAPY

- ☐ Immune inflammation of lungs, bowel, liver, kidneys, glands, skin, heart, nerves or muscles
- ☐ Infusion reactions; injection-site effects if given under the skin
- ☐ Some effects need hospital treatment, steroids or long-term hormone replacement
- ☐ Effects can begin after the final dose

Ipilimumab
IMMUNOTHERAPY

- ☐ Immune colitis and diarrhoea
- ☐ Immune liver, hormone, skin, nervous-system and other organ inflammation
- ☐ Serious or potentially life-threatening reactions
- ☐ Some adverse effects persist after the four-dose phase

Discuss the exact colorectal setting and two treatment phases, expected benefit and uncertainty, alternatives, greater immune-toxicity risk with the combination, possible hospital care, steroids, lasting hormone replacement and rare life-threatening effects. Explain late toxicity, pregnancy precautions, the contact plan and the option to reconsider.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00551 v7a.

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